A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Leila Byrne
Turn The Corner Medical Clinic

Dear Dr Byrne,

Re: Ms Emma Fallon
DOB: 1993-03-31
Address: 2 O'Keefe Street, BELLFIELD VIC 3081

Summary

- **Gastroscopy and colonoscopy revealing no evidence of Crohn's disease, coeliac disease, or Helicobacter pylori infection.**
- **Excision of two benign polyps, including a small adenoma requiring five-year surveillance colonoscopy.**
- **Resolution of energy levels and brain fog following intravenous iron replacement.**
- **Persistent loose stools likely secondary to metformin use; for trial of fibre supplementation.**
- **Action required: Please monitor iron studies over the next six months. If levels decline rapidly despite dietary optimisation, please refer her back for a small bowel capsule endoscopy.**

I had the pleasure of conducting a review with Emma following her gastroscopy and colonoscopy performed at St Vincent's Hospital. I am pleased to report that the investigations were largely reassuring and have helped clarify the management of her iron deficiency and gastrointestinal symptoms.

The gastroscopy was macroscopically normal. Biopsies of the small bowel and stomach were negative for coeliac disease, Crohn's disease, and *Helicobacter pylori*. These results effectively exclude malabsorption or gastric infection as the cause of her iron deficiency.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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The colonoscopy revealed two small polyps which were successfully excised. Histological examination confirmed both were benign; however, one polyp possessed pre-cancerous potential. Based on the size and histological features, Emma will require a surveillance colonoscopy in five years' time. I have placed her on my clinical recall system for this.

Regarding her clinical status, Emma reports a significant improvement in her fatigue and cognitive "brain fog" following the iron infusion administered post-procedure. We discussed the likely cause of her anaemia, noting that as no gastrointestinal source of bleeding or malabsorption was identified, her iron loss is likely attributable to historical menstrual loss and dietary factors. Her bowel habits remain loose and frequent, which she notes exacerbated upon increasing her metformin dose to 1.5 grams.

Impression and Plan Emma's gastrointestinal symptoms are almost certainly a side effect of metformin. She is currently consulting with a dietitian and plans to introduce a fibre supplement to help regulate stool consistency. We have agreed to pursue these dietary interventions rather than introducing further medications at this stage. Should these measures prove insufficient, a modest reduction in her metformin dose may be considered.

Regarding her iron deficiency, the probability of significant small bowel pathology is very low. Consequently, I do not believe a small bowel capsule endoscopy is warranted at this juncture. However, if her iron stores deplete rapidly over the coming six months despite an appropriate diet, I would be happy to facilitate a capsule endoscopy through the public hospital system. I have advised Emma to contact me if she develops any acute symptoms or if her bowel habit becomes unmanageable despite the current plan.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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